Eye Injuries

| Guideline Title: Eye Injuries | Guideline Number: GUID-3203-T006 |
|---|---|

| | |
|---|---|
| Issue date: 09/09/2014 | Replaces Dept. Policy: |
| Revision dates: 6/8/2021 | Developed by: Air Care Team |
| Approved by: Christopher Hunter, MD<br>Air Care Team Medical Director | Approved by: |
| Signature: | Signature: |
| Department Numbers: 3203 | |

CXI. **PURPOSE:** To recognize eye injury or potential for eye injury and provide rapid transport to the nearest appropriate facility while preventing any further damage from occurring.

CXII. **PATHOPHYSIOLOGY:** Eye injuries can involve any portion of the eye or its surrounding structures. The pupils may become irregularly shaped because of a ruptured globe or prolapse of the iris. It is important to assess for traumatic hyphema or asymmetric proptosis (bulging of the globe).

CXIII. **DEFINITIONS:**

CXIV. **DEPARTMENT GUIDELINE:**

A. If shield is already in place, make sure no pressure is being applied to globe itself.
B. If available, apply appropriate rigid shield over affected eye. Consider patching or shielding unaffected eye for comfort and to minimize movement.
C. If chemical injury exists, irrigate eye for 15-30 minutes with normal saline.
D. Remove superficial, non-impacted foreign bodies from the eyelids. Do not attempt to remove any intraocular foreign bodies.
E. Stabilize any impaled object with gauze and cling to prevent movement. Do no remove any impaled object. Consider covering the unaffected eye to minimize movement.
F. Avoid eye manipulation as much as possible
G. An open globe injury is a relative contraindication for use of succinylcholine, if patient requires rapid sequence intubation rocuronium is recommended for paralysis
H. Utilize antiemetics to avoid increased intraocular pressure caused by vomiting.
I. If hyphema exists and there are no contraindications, transport with head of bed elevated to at least 45 degrees
J. During flight, maintain as low an altitude as is safely possible

CXV. **DOCUMENTATION:** EMS Charts

CXVI. **REFERENCES:**